

Enhancing Operational Efficiency and Patient Experience at HealthFirst Care



Name: Sajjad Karimian

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Executive Summary

Problem:

1. HealthFirst Care faces long patient wait times, inefficient resource allocation, and communication gaps that are reducing patient satisfaction and operational efficiency.

Key insights:

2. Data confirms the scale of the problem: a 67.5% appointment non-completion rate, a sharp 6 PM scheduling peak, and 13.6% of patient feedback rated Low satisfaction.
3. Orthopedics and General Medicine show the weakest patient satisfaction and resource performance, making them priority departments for improvement.

Recommended actions:

4. Fast-track automated scheduling and real-time resource visibility as the two highest-impact, lowest-risk improvements.
5. Prioritize Orthopedics and General Medicine for targeted wait-time and communication improvements.
6. Maintain the phased rollout with UAT sign-off and documented contingency plans for all 7 High-priority risks.

Introduction

Opportunity:

1. HealthFirst Care can meaningfully improve patient experience and operational efficiency by addressing scheduling, resource, and communication inefficiencies.
2. Data-driven analysis of appointment, feedback, and resource records reveals specific, addressable root causes rather than vague operational complaints.

Approach:

3. This project followed a structured business-analysis lifecycle: requirements gathering, stakeholder engagement, process mapping, data analysis, and risk management.
4. Each phase builds on the last, from the BRD and RTM through to a risk-assessed, phased implementation plan.

Key questions/hypotheses:

5. Where in the patient journey do the greatest delays and dissatisfaction occur, and which departments are over- or under-utilized relative to demand?
6. Hypothesis: scheduling and check-in inefficiencies are the primary drivers of long wait times and lower patient satisfaction, more so than raw resource shortages.

Business Objectives

1. Reduce average patient wait times by 20% within 12 months of go-live.
2. Reduce double-booking and scheduling-conflict incidents through automated conflict detection.
3. Improve inter-departmental communication and handoff efficiency between clinical and administrative teams.
4. Improve overall patient satisfaction, with particular focus on Orthopedics and General Medicine.
5. Optimize resource allocation across departments to reduce peak-hour bottlenecks.
6. Eliminate data silos between scheduling and records systems, within the approved budget.



Methodology

Requirements Gathering: Business Requirement Document (BRD)

Problem statement:

- HealthFirst Care faces long patient wait times, inefficient resource allocation, and communication gaps across departments. These operational challenges are reducing patient satisfaction and straining staff capacity, creating an urgent need for a structured, data-driven improvement initiative.
- **Key requirements to improve operational efficiency:**
 1. Automated appointment scheduling with real-time conflict detection (FR1).
 2. Automated patient notifications for confirmations, changes, and delays (FR2).
 3. Real-time resource visibility across departments to reduce peak-hour bottlenecks (FR3).
 4. Structured inter-departmental handoff/referral workflow with status tracking (FR4).
 5. Integration between the scheduling module and existing patient records system (FR5).
 6. Operational reporting dashboard tracking wait times, utilization, and satisfaction (FR6).

Requirements Gathering: Business Requirement Document (BRD)

Constraints:

- 1. Budget limits the project to system integration rather than full replacement of legacy systems.
- 2. Implementation must be phased to avoid disrupting ongoing patient care.
- 3. The solution must operate within the existing hospital network infrastructure, which currently experiences frequent downtime.
- 4. Applicable data-protection requirements (e.g., HIPAA or the equivalent local regulation) must be met for all patient data handled by the new system.

- **Acceptance criteria:**

- 1. Reduce average patient wait times by 20% within 12 months of go-live.
- 2. Reduce double-booking and scheduling-conflict incidents.
- 3. Achieve measurable improvement in patient satisfaction scores, particularly in Orthopedics and General Medicine.
- 4. Successfully integrate scheduling and patient records systems with no critical data loss.

Requirements Gathering: Requirement Traceability Matrix (RTM)

Requirement ID	Requirement Description	Priority (MoSCoW)	Stakeholder(s)	Project Objective	Related Data File	Status
FR1	Automate scheduling with real-time availability and conflict detection.	Must Have	Admin Staff; Patients	Reduce wait times and scheduling errors	appointment_data.csv	Approved
FR2	Automatic patient notifications (email/SMS) for confirmations, changes, delays.	Must Have	Patients; IT Team	Improve patient communication and satisfaction	feedback_data.csv	Approved
FR3	Real-time view of equipment, room, and staff availability.	Must Have	Doctors; Nurses	Improve resource allocation efficiency	resource_data.csv	Approved
FR4	Structured inter-departmental handoff/referral workflow.	Should Have	Doctors; Nurses	Close communication gaps across departments	Stakeholder Profiles	Pending
FR5	Integrate scheduling module with existing patient record system.	Must Have	IT Team	Eliminate data silos across systems	Stakeholder Profiles	Approved
FR6	Report on wait times, resource utilisation, and patient satisfaction.	Should Have	Sponsor; Admin Staff	Track progress toward business objectives	appointment/feedback data	Pending

Requirements Gathering: Requirement Traceability Matrix (RTM) — continued

Requirement ID	Requirement Description	Priority (MoSCoW)	Stakeholder(s)	Project Objective	Related Data File	Status
NFR1	Scheduling interface simple for patients, efficient for staff.	Must Have	Patients; Admin Staff	Improve patient satisfaction and adoption	feedback_data.csv	Approved
NFR2	Minimise network downtime affecting scheduling/communication.	Must Have	IT Team	Ensure reliable system availability	Stakeholder Profiles	Approved
NFR3	Remediate security vulnerabilities; protect patient data.	Must Have	IT Team	Ensure data security and regulatory compliance	Stakeholder Profiles	Approved
NFR4	Real-time features update without delay at peak times.	Should Have	Admin Staff; Patients	Maintain performance during peak demand	appointment_data.csv	Pending
NFR5	Solution should be cloud-capable for future scaling.	Could Have	IT Team	Support future scalability	Stakeholder Profiles	Pending

Stakeholder Analysis and Engagement Plan

Stakeholders:

- Patients, Doctors & Nurses, Administrative Staff, IT Teams, and Hospital Leadership.
- **Stakeholders' influence:**
- Hospital Leadership — High influence (approves budget and scope; accountable for outcomes).
- Doctors & Nurses — High influence (delivery of care depends on their adoption).
- Administrative Staff — High influence (operate scheduling, records, and billing daily).
- IT Teams — High influence (own technical architecture, integration, and security).
- Patients — Low formal influence, though directly affected day-to-day.

Stakeholder Analysis and Engagement Plan

Stakeholder engagement strategies:

- Hospital Leadership: weekly steering-committee meetings with detailed progress reports.
- Doctors & Nurses, Administrative Staff (Key Players): weekly meetings, hands-on UAT involvement, and department champions.
- IT Teams (Keep Satisfied): bi-weekly email status updates on technical progress.
- Patients (Keep Informed): monthly newsletters, dashboards, and satisfaction surveys.
- **Stakeholder communication strategies:**
 - Steering-committee meetings and status reports for Leadership and IT.
 - Clinical advisory meetings and UAT sessions for Doctors, Nurses, and Administrative Staff.
 - Email, patient portal, and surveys for Patients.

Scope Management Plan

In-scope activities:

- Automated appointment scheduling with real-time conflict detection.
- Automated patient notifications (email/SMS) for confirmations, changes, and delays.
- Real-time resource visibility across departments.
- Integration between scheduling and patient records systems.
- Operational reporting dashboard and inter-departmental handoff workflow.

• Out-of-scope activities:

- Replacement of the core Hospital Information System (HIS) or billing platform.
- Physical facility expansion (e.g., new operating rooms, additional beds).
- Telemedicine/virtual-consultation capability and staffing-level changes.

Scope Management Plan

Assumptions:

- Stakeholders are available for reviews and sign-off at each project milestone.
- The appointment, feedback, and resource data provided are representative of recurring operational patterns.
- Existing IT infrastructure can support integration without a full core-system replacement.

- **Constraints:**

- Budget limits the project to system integration rather than replacement.
- Rollout must be phased to avoid disrupting ongoing patient care.
- The solution must operate within the existing, occasionally unreliable, hospital network.

Scope Management Plan

Phases in the Work Breakdown Structure (WBS):

WBS ID	Task Name	Task Description	Milestone
1.1	Phase 1: Requirements Gathering & Stakeholder Analysis	Define and document project requirements and stakeholder needs.	BRD & Stakeholder Engagement Plan
1.2	Phase 2: Scope Definition	Define and document project scope.	Scope Management Plan
1.3	Phase 3: Process Analysis & Redesign	Map current and future-state workflows.	Process Models (As-Is/To-Be)
1.4	Phase 4: Solution Design & Development	Build the scheduling, notification, and integration solution.	Functional system build complete
1.5	Phase 5: Reporting & Dashboard Development	Build KPI reporting dashboard.	Dashboards and notification systems live
1.6	Phase 6: Testing & Validation	Test and validate the solution.	UAT sign-off
1.7	Phase 7: Deployment & Training	Roll out the solution in phases.	Phased go-live complete
1.8	Phase 8: Project Closure & Final Presentation	Close out the project and report results.	Final presentation delivered

Scope Management Plan

Scope change management:

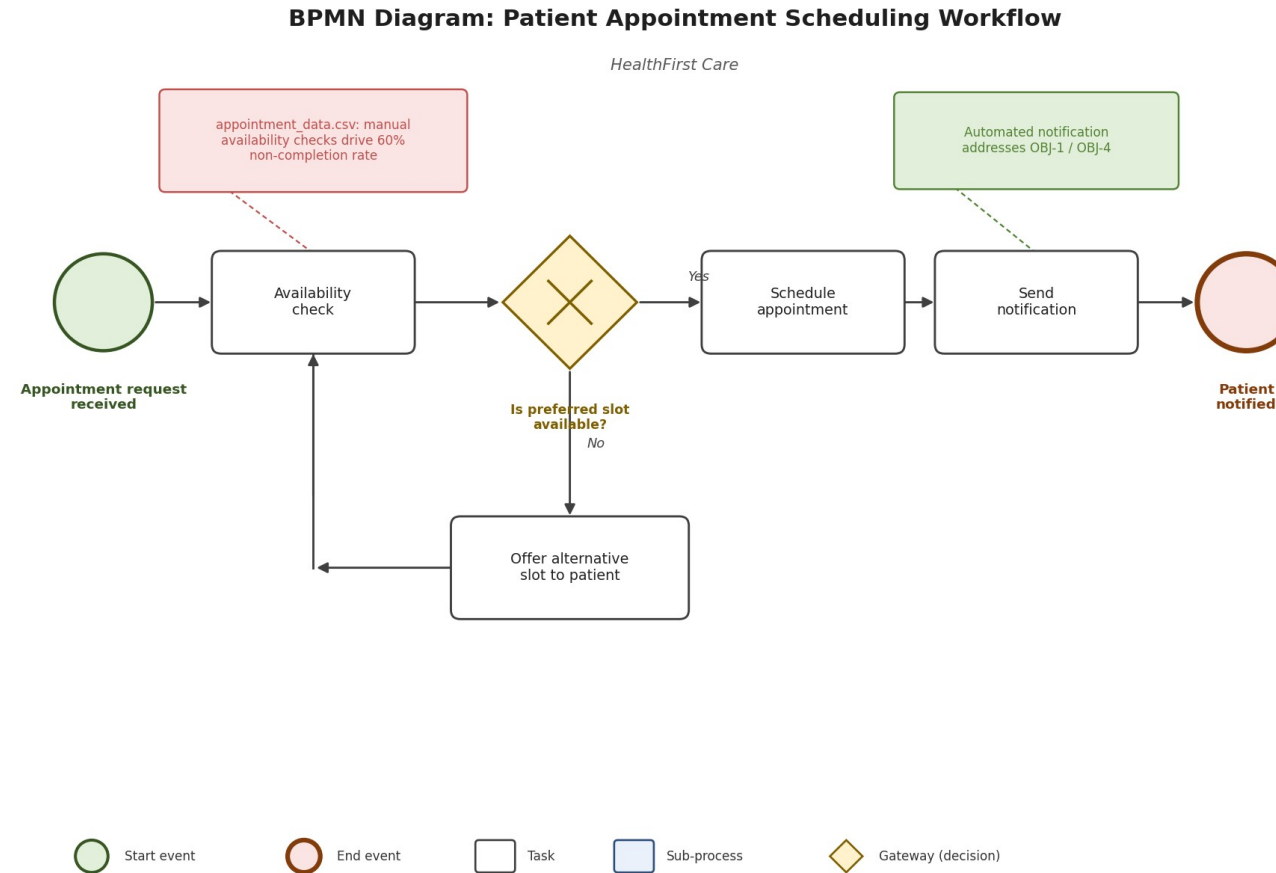
- Any proposed change is submitted as a Scope Change Request, documenting the change, rationale, and affected requirement ID(s).
- The Business Analyst and IT Manager assess impact on scope, schedule, and budget before review.
- Changes are approved by the Project Sponsor before the BRD, RTM, and WBS are updated and communicated to affected stakeholders.

Process Mapping

Process	As-Is Model	To-Be Model
Appointment Scheduling	Manual booking with no conflict detection; 60%+ of appointments end in No Show, Rescheduled, or Cancelled.	Automated scheduling with real-time conflict detection and instant SMS/email confirmation (FR1, FR2).
Patient Check-In	Paper intake forms, manual re-keying into records, and long unpredictable queue waits.	Self-service kiosk/app check-in with automated verification and instant department notification.
Interdepartmental Communication	Untracked phone/email requests between Admin Staff and IT, with no audit trail.	Centralized task-management dashboard with automated alerts and closed-loop confirmation (FR3, FR4).

Advanced Process Mapping

Detailed workflow using the advanced BPMN model:



Advanced Process Mapping

Stakeholder responsibility using the Swimlane diagram:

Swimlane (Stakeholders)	Task/Activity	Description
Patient	Request appointment	Submits appointment request via online portal/app.
Administrative Staff	Check doctor availability	Manually checks doctor/resource availability (current bottleneck).
Administrative Staff	Enter scheduling details	Enters appointment details into the scheduling system.
IT Team	Resolve system issues	Resolves system errors when the scheduling process fails.
Administrative Staff	Confirm booking	Confirms the appointment booking with the patient.
Patient	Receive confirmation	Receives appointment confirmation notification.
Front Desk	Check-in patient	Verifies patient identity and appointment (current paper-based process).
Nurses	Assess resource needs	Assesses room/equipment needs for the patient visit.
Nurses	Escalate resource shortage	Escalates and waits when a required resource is unavailable.
Doctors	Conduct consultation	Conducts the patient consultation once resources are confirmed.

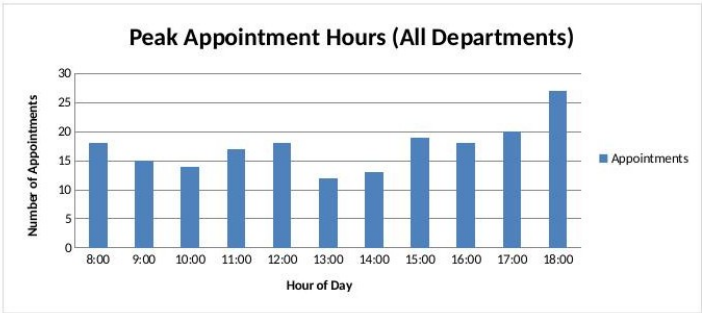
Data Analysis

Trends using a Pivot Table:

Appointments per Department & Hour of Day											
Department	8	9	10	11	12	13	14	15	16	17	18
Cardiology	5	2	0	3	6	2	4	7	2	4	2
General Medicine	3	2	2	2	2	3	2	1	3	2	4
Neurology	3	1	5	4	6	0	2	4	2	2	5
Oncology	1	3	1	3	2	2	0	3	6	3	5
Orthopedics	2	2	1	3	1	2	2	4	2	5	6
Pediatric	4	5	5	2	1	3	3	0	3	4	5

Total Appointments by Hour of Day (all departments)

Hour	Appointments
8:00	18
9:00	15
10:00	14
11:00	17
12:00	18
13:00	12
14:00	13
15:00	19
16:00	18
17:00	20
18:00	27



Pivot table data: appointments peak at 18:00 (27), with secondary peaks at 17:00 (20) and 15:00 (19); the quietest hour is 13:00 (12). Totals are broken down by department and hour.

Data Analysis

Trends analyzed from the Pivot Table:

- Appointments peak sharply at 6:00 PM (27 appointments), well above every other hour, with secondary peaks at 5:00 PM (20) and 3:00 PM (19).
- Midday (1:00–2:00 PM) is the quietest period, suggesting a predictable lunch-hour dip in bookings.
- Only 28% of all appointments are marked Completed; the remaining 72% are split across No Show, Rescheduled, and Cancelled outcomes.
- The non-completion pattern is consistent across departments, indicating a systemic scheduling issue rather than one department's problem.

Data Analysis

Key insights:

- Orthopedics has the highest share of Low patient-satisfaction feedback (27.6%), despite the lowest resource utilization rate (33.7%) — a process problem, not a capacity problem.
- General Medicine shows the heaviest equipment/room maintenance backlog, while Pediatric and Oncology show the most staff/doctor unavailability.
- 86.4% of feedback responses are High satisfaction overall, showing the problem is concentrated in specific departments rather than hospital-wide.
- The 6 PM scheduling peak and the 67.5% non-completion rate are directly linked: automating conflict detection should address both simultaneously.

Data Visualization

Average patient wait time using a horizontal bar chart:

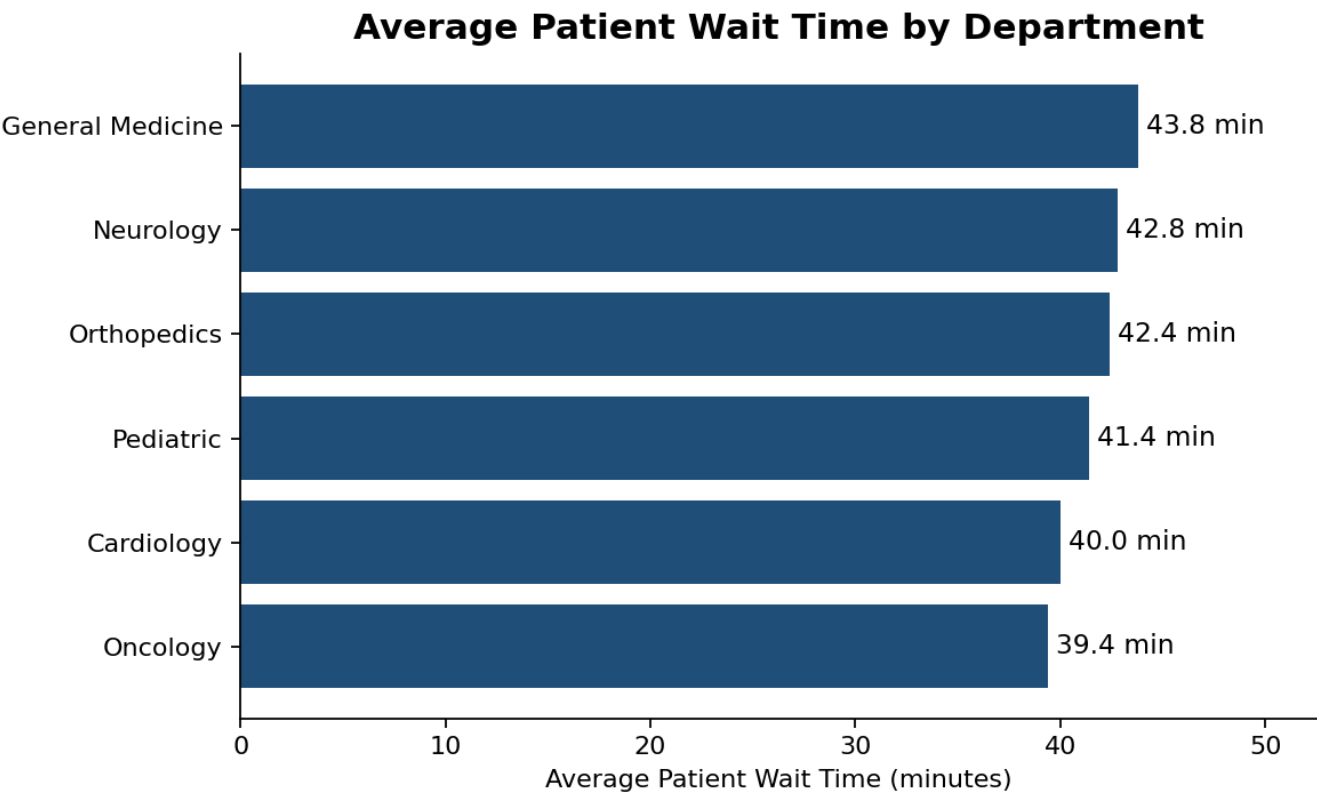


Chart data: General Medicine has the highest average wait time (43.8 min), followed by Neurology (42.8), Orthopedics (42.4), Pediatric (41.4), Cardiology (40.0), and Oncology (39.4, lowest).

Data Visualization

Bar chart highlighting overused and underutilized resources:

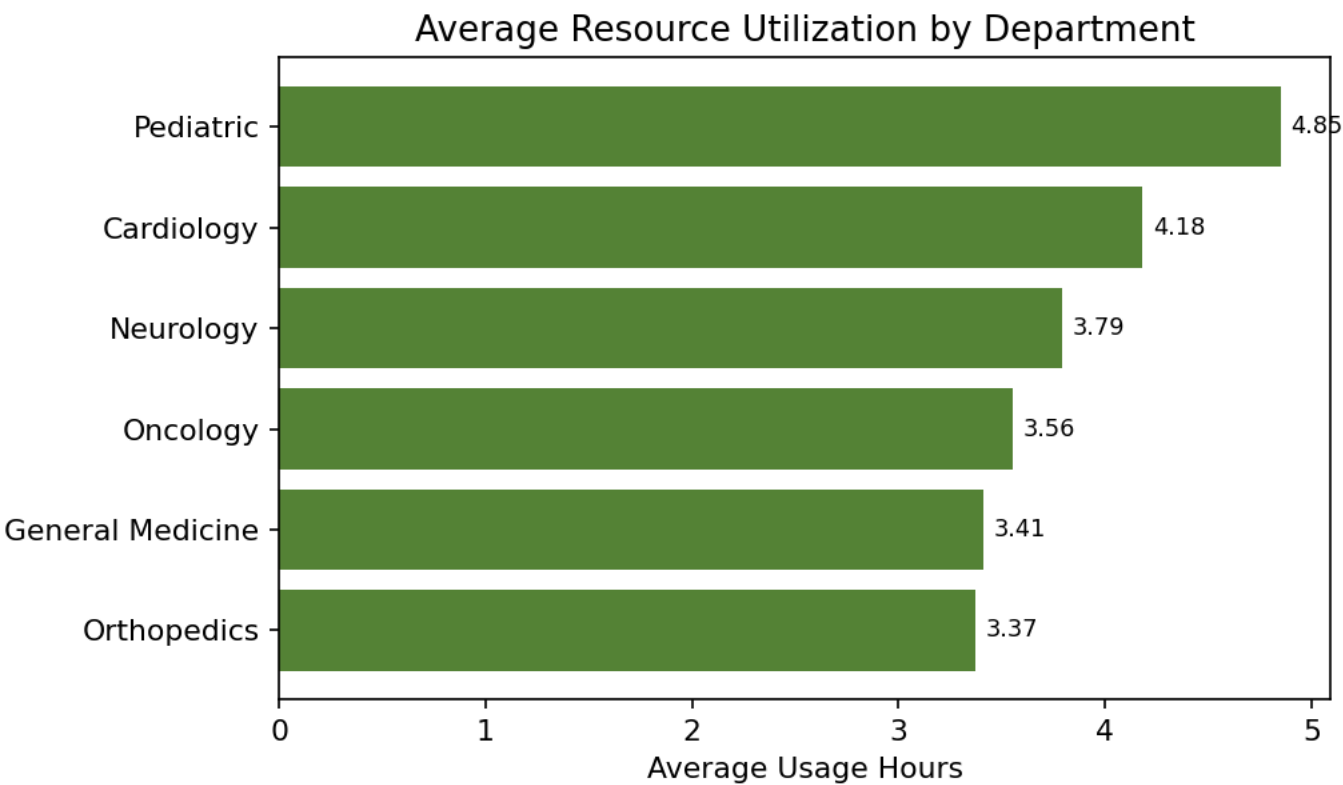


Chart data: Pediatric has the highest resource utilization rate (48.5%, overused); Orthopedics has the lowest (33.7%, underutilized). Cardiology, Neurology, Oncology, and General Medicine fall in between.

Data Visualization

Patient feedback visualized using a Pie Chart:

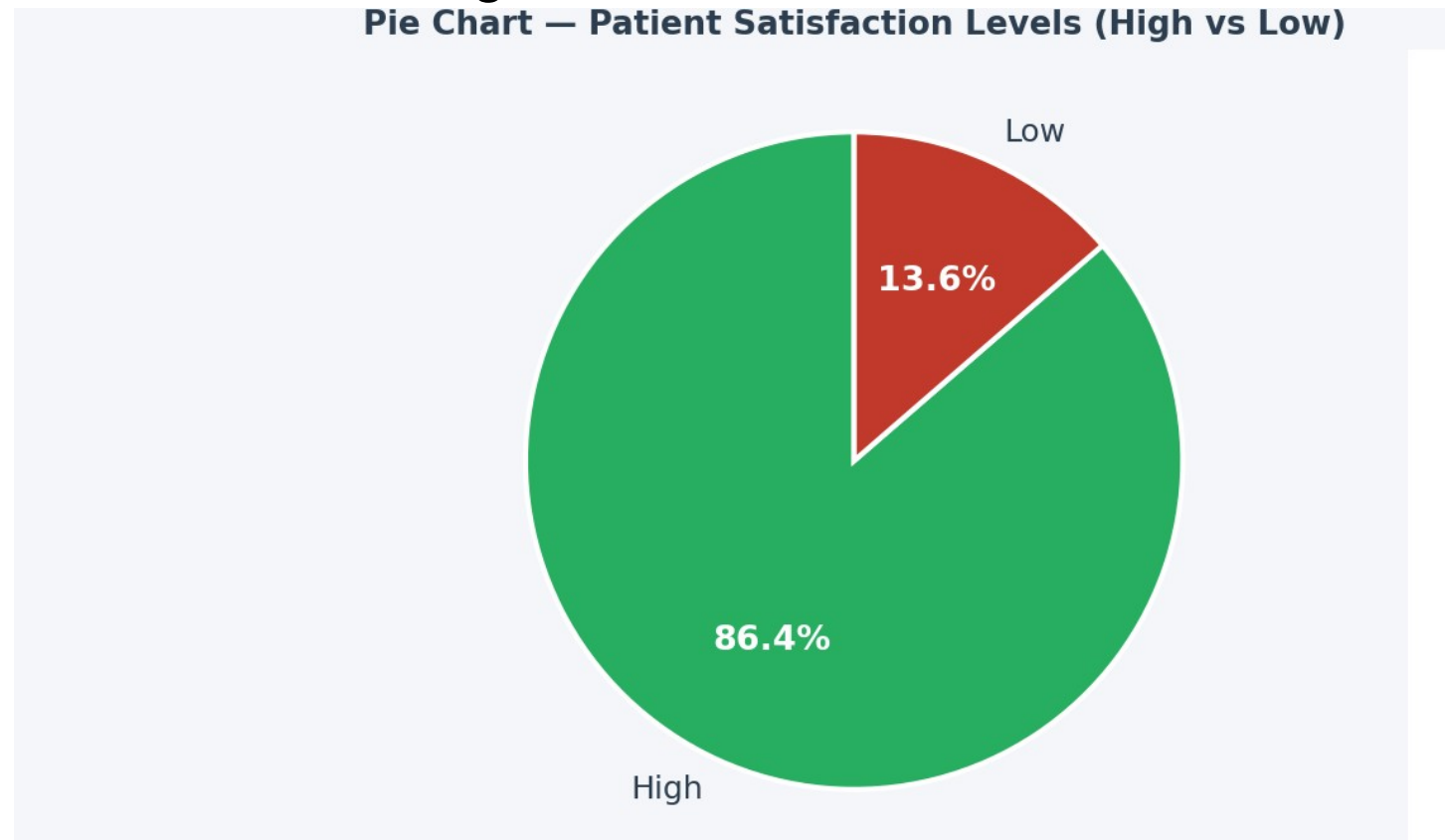


Chart data: 86.4% of patient feedback responses are High satisfaction; 13.6% are Low satisfaction.

Data Visualization

Heat Map showing the efficiency of departments:

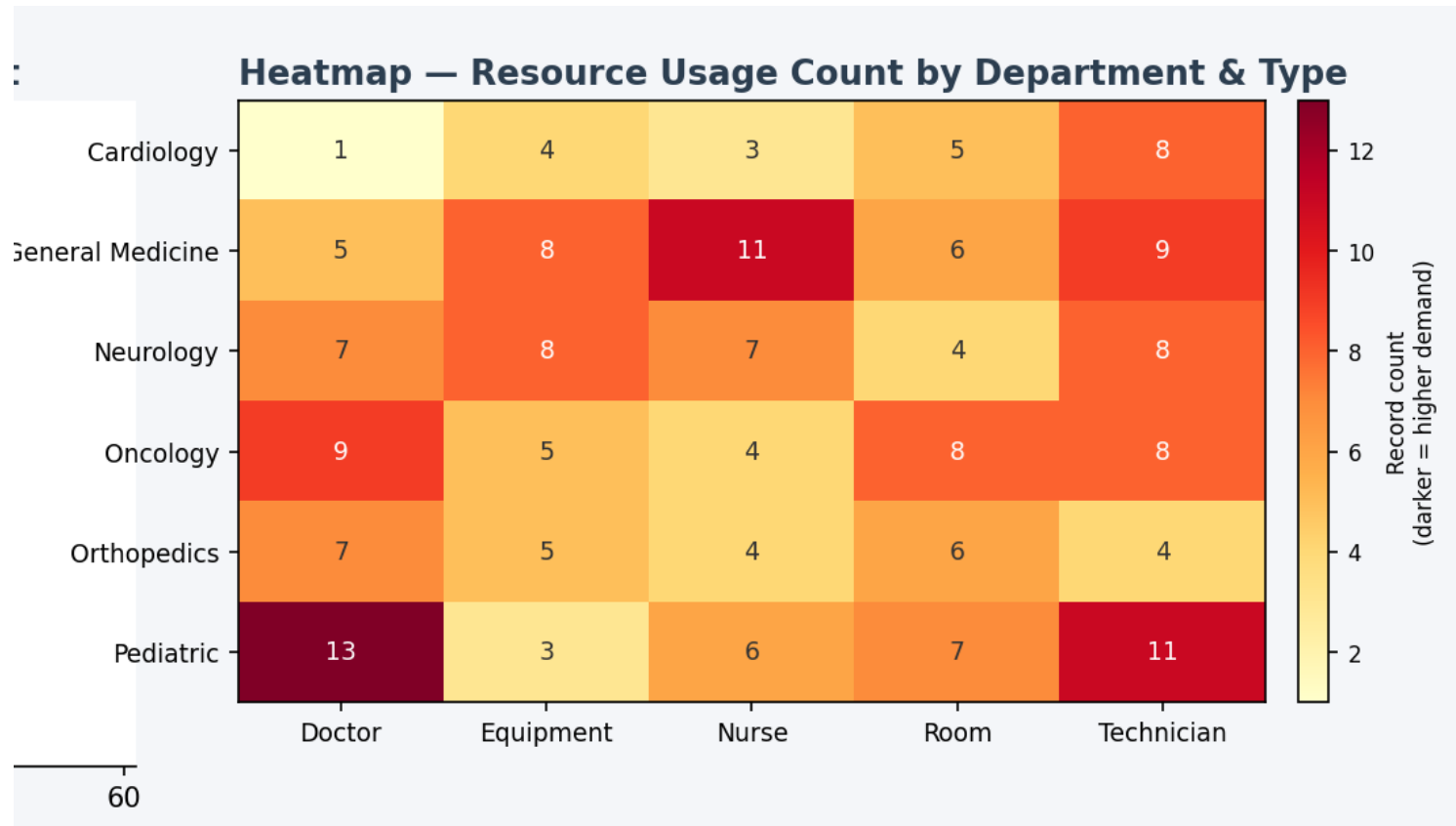


Chart data: Pediatric shows the heaviest resource demand (13 Doctor records, 11 Technician records); General Medicine shows the highest single concentration (11 Nurse records), indicating department-specific bottlenecks.

Risk Management Plan

Risks identified in the risk register:

Risk ID	Risk Description	Category	Likelihood	Impact	Severity	Mitigation Strategy
OP-1	Workflow disruption during phased system rollout.	Operational	Medium	High	High	Phased WBS rollout with UAT sign-off and rollback plan.
OP-2	Staff resistance to new automated workflows.	Operational	High	Medium	High	Role-specific training; department champions; weekly feedback.
OP-3	Double-booking continues before FR1 fully adopted.	Operational	Medium	Medium	Medium	Prioritize FR1/FR5 early; track completion rate weekly.
OP-4	Resource shortages during go-live create bottlenecks.	Operational	Medium	High	High	Deploy FR3 dashboard; cross-department sharing protocol.
OP-5	Discharge/billing handoff delays reduce bed turnover.	Operational	Medium	Medium	Medium	Implement FR4 handoff workflow before billing is requested.
TE-1	Data breach or security vulnerability.	Technical	Medium	High	High	Full security audit; encryption; sign-off gate before go-live.
TE-2	System/network downtime disrupts operations.	Technical	High	Medium	High	Implement NFR2 reliability requirements and failover plan.
TE-3	Integration failure duplicates or corrupts data.	Technical	Medium	High	High	Dedicated UAT before Phase 2; parallel reconciliation checks.

Risk Management Plan

Risks identified in the risk register (continued):

Risk ID	Risk Description	Category	Likelihood	Impact	Severity	Mitigation Strategy
TE-4	Legacy data-quality issues mislead decisions.	Technical	High	Medium	High	Apply data-cleaning standards; label known limitations.
TE-5	A reporting assumption mistaken for verified fact.	Technical	Medium	Low	Low	Require assumptions to be labeled and confirmed.
ST-1	Patients under-adopt the self-service kiosk.	Stakeholder	Medium	Medium	Medium	Retain assisted check-in alongside the kiosk.
ST-2	Leadership and IT scope expectations misalign.	Stakeholder	Low	Medium	Low	Route gaps through Scope Change Management Process.
ST-3	Doctors/Nurses under-adopt the new handoff workflow.	Stakeholder	Medium	Medium	Medium	Apply manage-closely engagement plan during rollout.
ST-4	Sponsor change reduces Key Player engagement.	Stakeholder	Low	High	Medium	Maintain documented sponsor/steering-committee continuity.
ST-5	Compliance requirements surface late, delay rollout.	Stakeholder	Low	High	Medium	Confirm compliance requirements at project initiation.

Risk Management Plan

Risks categorized based on the Risk Assessment Matrix:

Likelihood/ Impact	Low Impact	Medium Impact	High Impact
High Likelihood	Medium (3) —	High (6) OP-2, TE-2, TE-4	High (9) —
Medium Likelihood	Low (2) TE-5	Medium (4) OP-3, OP-5, ST-1, ST-3	High (6) OP-1, OP-4, TE-1, TE-3
Low Likelihood	Low (1) —	Low (2) ST-2	Medium (3) ST-4, ST-5

Risk Management Plan

Elements identified in the SWOT analysis:

Strengths

- Established Key Player sponsorship (Leadership, IT engaged weekly).
- Traceable requirements: every BRD requirement linked to an objective in the RTM.
- Cleaned, structured operational data available for evidence-based decisions.

Weaknesses

- Heavy reliance on manual scheduling, paper check-in, and phone/email coordination.
- Data silos between scheduling and records systems.
- Budget constraint limits the project to integration, not full replacement.

Opportunities

- Automating scheduling/check-in directly targets the largest measured bottleneck.
- Centralized reporting can convert ad hoc communication into an auditable process.
- Phased rollout allows incremental proof-of-value before full investment.

Threats

- Legacy security vulnerabilities and network downtime already reported by IT.
- Staff/stakeholder resistance to changing entrenched manual habits.
- Regulatory/data-protection requirements not yet fully scoped.

Risk Management Plan

Key insights from the Risk Management Plan:

- Of 15 identified risks, 7 are High priority, spanning Operational, Technical, and Stakeholder categories in roughly equal measure.
- Delays and risk exposure cluster at handoffs between roles (e.g., Admin to IT, Nurses to Doctors) rather than within any single team's own workload.
- Security (TE-1) and downtime (TE-2) risks echo concerns IT stakeholders already raised directly, confirming they are not hypothetical.
- Mitigation must be coordinated across Operations, IT, and stakeholder engagement together — no single team can address all 7 High risks alone.

Risk Mitigation Plan

Strategies to mitigate risks:

Risk ID	Risk Description	Category	Likelihood	Impact	Severity	Mitigation Strategy
OP-1	Workflow disruption during phased rollout of the new scheduling/check-in system.	Operational	Medium	High	High	Follow the phased WBS rollout with UAT sign-off and rollback plan before each phase; increase Leadership/IT engagement during cutover weeks.
OP-2	Staff resistance to adopting new automated workflows.	Operational	High	Medium	High	Role-specific training for each stakeholder group; designate department champions; weekly feedback sessions during rollout.
OP-4	Resource shortages (nurses, rooms, equipment) during go-live.	Operational	Medium	High	High	Deploy the FR3 resource-visibility dashboard proactively; cross-department resource-sharing protocol for peak-demand windows.
TE-1	A data breach or exploited security vulnerability compromises patient data.	Technical	Medium	High	High	Full security audit and penetration test before go-live (NFR3); encryption enforced; formal security sign-off gate at Milestone M5.
TE-2	System/network downtime disrupts scheduling and dashboard availability.	Technical	High	Medium	High	Implement NFR2 reliability requirements and a documented failover plan; monitor uptime continuously as a KPI.
TE-3	Integration between scheduling and records systems (FR5) fails or duplicates data.	Technical	Medium	High	High	Sequence FR5 integration with dedicated UAT (Milestone M7) before Phase 2 go-live; run parallel reconciliation checks.
TE-4	Legacy data-quality issues mislead decision-making.	Technical	High	Medium	High	Apply documented data-cleaning standards as an ongoing checkpoint; label known lookup limitations on any dashboard that uses them.

Risk Mitigation Plan

Factors included in the Contingency Plan:

Risk ID	Contingency Plan
OP-1	Activate rollback to the prior manual process immediately; notify Leadership and affected departments within 1 hour; root-cause review within 24 hours.
OP-2	Escalate to Leadership for reinforced messaging; temporarily dual-run old and new processes; re-survey satisfaction after two more weeks.
OP-4	Activate cross-department resource-sharing protocol immediately; notify affected patients of any resulting delay; log the event for review.
TE-1	Isolate affected systems from the network immediately; notify IT Manager and Leadership within 1 hour; document the incident for compliance.
TE-2	Switch to the documented manual fallback process; IT provides hourly status updates; reconcile records after restoration.
TE-3	Revert to the pre-integration records process immediately; run a data-reconciliation script; delay the next milestone until re-tested.
TE-4	Flag the affected report to Leadership immediately; pause any pending decision; reissue the report with a documented correction note.

Risk Mitigation Plan

Risks prioritized based on the Visual Risk Matrix:

Priority Level	Risk ID & Description	Rationale	Action Urgency
High	OP-1: Workflow disruption during phased rollout	Directly threatens continuity of patient care during transition.	Immediate — gate at each WBS milestone
High	OP-2: Staff resistance to new workflows	High likelihood given entrenched manual habits; blocks adoption of every other improvement.	Immediate — address before Phase 1 go-live
High	OP-4: Resource shortages during go-live	Directly impacts patient throughput during peak hours.	High — monitor continuously via FR3 dashboard
High	TE-1: Data breach / security vulnerability	Severe impact on patient trust and regulatory compliance.	Immediate — mandatory go/no-go gate before go-live
High	TE-2: System/network downtime	IT already reports frequent outages; high likelihood of recurrence.	Immediate — failover plan before Phase 1
High	TE-3: System integration failure	Could corrupt patient records across two systems simultaneously.	High — dedicated UAT before Phase 2
High	TE-4: Legacy data-quality issues	Already observed directly in cleaned datasets; ongoing risk to decisions.	Ongoing — continuous data validation checkpoint

Risk Mitigation Plan

Key insights from the Risk Mitigation Plan:

- All seven High-priority risks now have both a proactive mitigation strategy and a reactive contingency plan.
- Every contingency plan assigns a first responder (IT Manager, Project Manager, Administrative Staff, or Leadership) and a first action.
- Three risks (TE-1, TE-2, TE-3) gate specifically on WBS milestones (M5, M7) — their mitigations are go/no-go criteria, not background activities.
- This plan should be reviewed at each WBS phase gate, and immediately after any contingency plan is invoked, to capture lessons learned.

A photograph of a laptop on a wooden table in a dimly lit cafe. The background is filled with warm, out-of-focus lights, creating a bokeh effect. The laptop screen displays a network diagram with blue nodes and connecting lines. A white mug is visible to the right of the laptop.

Findings and Recommendations

Key Findings

1. 6:00 PM is the single busiest appointment hour (27 appointments), well above every other time slot, and only 28% of all appointments are marked Completed.
- 2. Orthopedics has the highest share of Low patient-satisfaction feedback (27.6%), despite having the lowest resource utilization rate (33.7%) — indicating a process problem, not a capacity problem.
- 3. General Medicine shows the heaviest equipment/room maintenance backlog, while Pediatric and Oncology show the most staff/doctor unavailability — two distinct resource bottlenecks.
- 4. Delays consistently cluster at handoffs between roles (e.g., Admin to IT, Nurses to Doctors) rather than within any single team's own workload.
- 5. Of 15 identified project risks, 7 are High priority and span Operational, Technical, and Stakeholder categories equally, so mitigation must be coordinated across all three.

Key Recommendations

1. Fast-track automated appointment scheduling with real-time conflict detection to directly target the 6 PM peak and the 67.5% non-completion rate.
- 2. Deploy the real-time resource-visibility dashboard ahead of go-live to prevent shortages in Pediatric and General Medicine before they occur.
- 3. Prioritize Orthopedics and General Medicine for targeted wait-time and communication improvements, since both combine below-average satisfaction with above-average dissatisfaction.
- 4. Treat security remediation (NFR3) as a mandatory go/no-go milestone before any patient data migrates to the new system.
- 5. Maintain the phased WBS rollout with UAT sign-off and rollback readiness at each milestone to protect continuity of patient care.

Conclusion

- 1. HealthFirst Care's patient-experience and operational-efficiency challenges are well-defined, data-confirmed, and traceable to specific, addressable root causes.
- 2. The proposed phased solution — automated scheduling, resource visibility, and centralized communication — is scoped, requirements-traced, and fits the approved integration-only budget.
- 3. 15 project risks have been assessed and mitigated, with clear contingency plans defined for the 7 High-priority risks.
- 4. Success should be measured against the objectives already defined: 20% wait-time reduction, fewer double bookings, and improved patient satisfaction, particularly in Orthopedics and General Medicine.
- 5. With phased rollout and continued stakeholder engagement, HealthFirst Care is well-positioned to deliver measurable improvements within 12 months.

A P P E N D I X

Appendix

Note: Use this section to include supplementary materials, such as charts, graphs, data tables, and other supporting documents, for this Business Analysis (BA) report.

- Supporting materials for this report include: the full Business Requirements Document (BRD), Requirements Traceability Matrix (RTM) workbook, Stakeholder Analysis and Engagement Plan, Scope Management Plan with WBS workbook, As-Is/To-Be process diagrams, BPMN and Swimlane diagrams, Data Analysis workbook with pivot tables, the operational dashboard export, the Risk Register and Risk Assessment Matrix workbook, and the Risk Mitigation Plan with contingency tables.